

CENTRAL CALIFORNIA
EMERGENCY MEDICAL SERVICES
A Division of the Fresno County Department of Public Health

Manual	Emergency Medical Services Administrative Policies and Procedures	Policy Number 530.30
Subject	Paramedic Treatment Protocols CHILDBIRTH	Page 1 of 3
References	Title 22, Division 9, Chapter 3.3 of the California Code of Regulations	Effective Fresno County: 01/15/82 Kings County: 04/10/89 Madera County: 06/15/85 Tulare County: 04/19/05

STANDING ORDERS	
Oxygen	Titrate to SpO ₂ > 94%
IV Access	LR TKO – standard tubing if complications or prolonged labor.
Transport	Early transport is indicated unless delivery is imminent (i.e. crowning, urge to push).
Contact Hospital	Per EMS Policy #530.02.
Routine Delivery of Newborn	
Control head descent	Cup hand over head and apply gentle counterpressure to prevent explosive delivery. If head is not presenting part, refer to Breech Delivery section below.
Check for nuchal cord	If cord is looped around the neck, gently slip cord over head or across shoulder. If cord is tight around neck and preventing descent, double clamp and cut between clamps.
Deliver anterior shoulder	Once the head is delivered, gently lower the head to deliver the anterior (upper) shoulder. If anterior shoulder does not deliver, refer to Shoulder Dystocia section below.
Complete delivery	Gently raise the head to deliver the posterior shoulder. The remainder of the body should then deliver smoothly. Hold the baby in a slightly head down position.
Cut cord	Clamp cord and cut at least 6 inches from newborn. Clamp and cut immediately if neonatal resuscitation is needed. Delay clamping 60 seconds for normal healthy birth.

STANDING ORDERS – CONTINUED ON NEXT PAGE

Approved By	Daniel J. Lynch (Signature on File at EMS Agency)	Revision
EMS Director		07/01/2026
EMS Medical Director	Miranda Lewis, MD (Signature on File at EMS Agency)	

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STANDING ORDERS (CONTINUED)	
Assess newborn	Assess fetal heart rate and respirations. If fetal heart rate < 100 BPM, gasping, or apnea refer to Neonatal Resuscitation Protocol (Policy #530.31).
Dry and warm newborn	If no resuscitation is needed, warm baby and place on mother's abdomen/chest skin to skin. Routine suctioning is not recommended and should be reserved for babies with airway obstruction or for those needing resuscitation.
Placenta delivery	When placenta delivers, transport it in a plastic bag. Do not apply traction on cord in attempts to deliver placenta. Do not delay transport for delivery of the placenta.
Fundal massage	Massage the mother's lower abdomen (uterine fundus). If post-partum hemorrhage present, see Post-Partum Hemorrhage section below.
Complications of Delivery	
<i>Complicated deliveries often require neonatal resuscitation. Call for additional resources early.</i>	
<u>PROLAPSED CORD</u>	
Protect cord	With a gloved hand, gently elevate presenting part to take pressure off the cord. Do not attempt to push cord back. Cover exposed portion of cord with saline soaked gauze. Continuously monitor the cord for the presence of a pulse.
Position patient	If pulsations cannot be restored with the maneuvers above, place mother in knee chest position or extreme Trendelenburg position with hips elevated.
<u>SHOULDER DYSTOCIA</u>	
McRoberts maneuver	Hyperflex both the mother's knees tightly to the chest.
Suprapubic pressure	If McRoberts maneuver fails to relieve the dystocia, apply firm suprapubic pressure downward and toward the side the fetal head is facing. Do not apply pressure to the uterine fundus. Do not apply excessive traction to the fetal head or neck.
<u>BREECH PRESENTATION</u>	
Deliver newborn	Support the presenting part and allow the newborn to deliver. Avoid compression of the cord by the presenting part. If the head does not deliver spontaneously, place a gloved hand into vagina with the palm facing the baby's face to form a "V" with your fingers on either side of the baby's nose and push the vaginal wall away from the baby's face until the head is delivered.
<u>POST-PARTUM HEMORRHAGE (>500 ML BLOOD LOSS AFTER DELIVERY)</u>	
Fundal massage	Firmly massage the lower abdomen (uterine fundus).
IV access	Establish two large bore IVs.
Fluid bolus	1000 ml Normal Saline or Lactated Ringers
Tranexamic acid	If < 3 hours from delivery with ongoing hemorrhage and systolic blood pressure < 90 mmHg: Administer 2 g TXA in 100 ml NS over 10 minutes

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SPECIAL CONSIDERATION AND PRIORITIES

1. If patient is in labor and is pushing, discreetly inspect the perineum (digital exams are not done). If crowning, prepare to deliver at the scene.
2. If inspection reveals a prolapsed cord or an abnormal presentation (foot, buttocks, hand or face): Transport immediately. The rare exception may be when transport time is long and delivery is far advanced (e.g., body delivered to beyond umbilicus). In this situation, the only hope for infant survival may be a physician directed delivery at the scene. Contact Base Hospital immediately.
3. Newborns are susceptible to rapid heat loss and must be kept warm and dry.
4. If multiple births are expected, clamp and cut cord of the first newborn prior to the next birth. Maintain the identity of the first born.
5. Light bleeding and blood-tinged fluid is expected during normal childbirth. Excessive bleeding during active labor may occur with placenta previa or placental abruption. Do not place hand/fingers into the vagina of a bleeding patient except in cases of a prolapsed cord or breech delivery that is not progressing. Immediate transport and treatment for hypovolemia, as needed, is indicated.
6. In the event of maternal cardiac arrest at >20 weeks gestation, transport immediately to the closest facility with CPR in progress. If available, a dedicated provider should be tasked with performing manual left uterine displacement throughout the resuscitation. Refer to EMS Policy #530.04.
7. Consider preeclampsia in any patient ≥ 20 weeks gestation and up to 6 weeks after delivery. A systolic blood pressure ≥ 140 mmHg or a diastolic blood pressure ≥ 90 mmHg are considered abnormal and should raise suspicion for preeclampsia.
8. Document time of delivery of infant and placenta, who performed the delivery, and cutting of cord. Document at what point in the delivery the paramedic took charge.